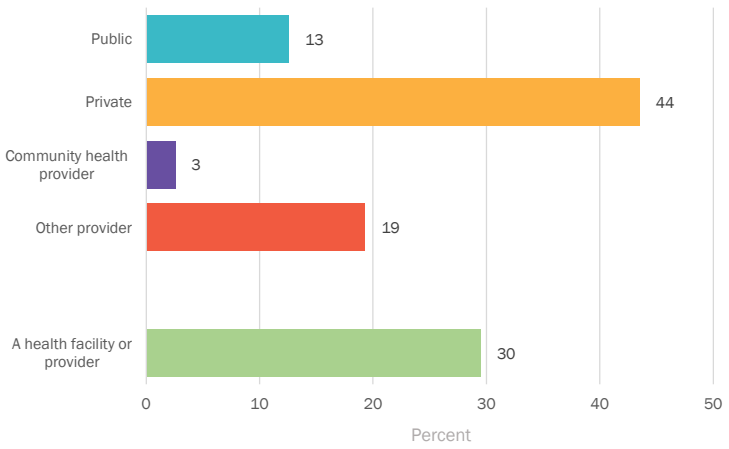
Bangladesh 2019

Child Health & Care of Illness

Multiple Indicator Cluster Surveys

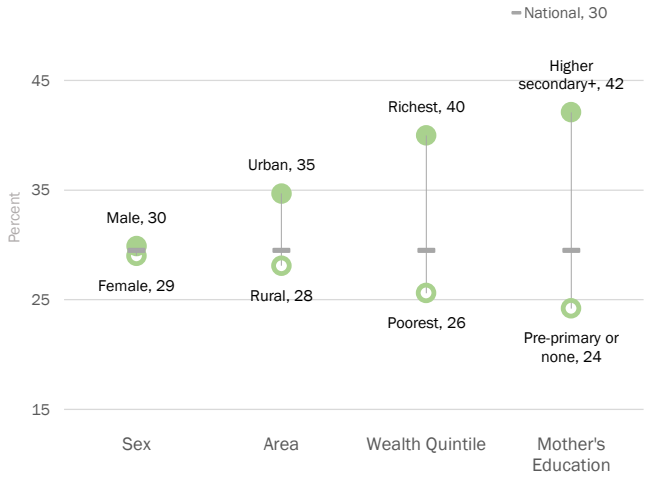
Diarrhoea

Care-seeking for Diarrhoea



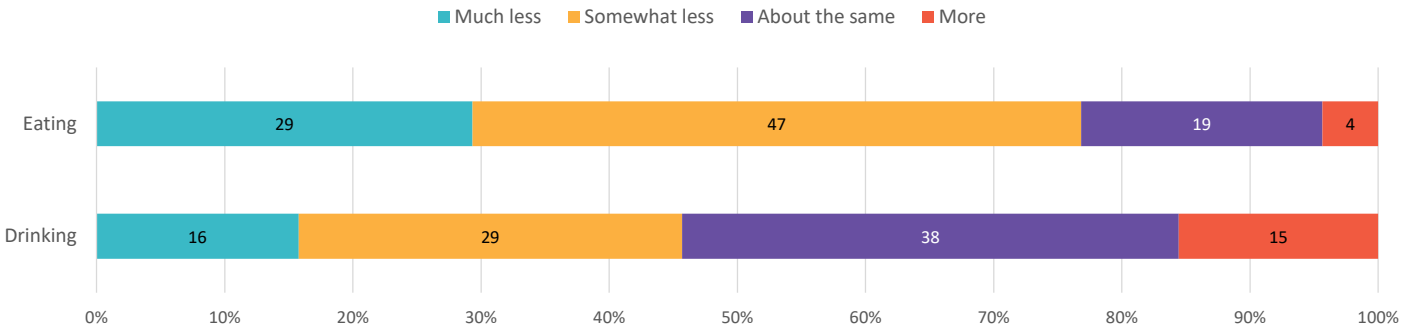
Percentage of children age 0-59 months with diarrhoea in the last two weeks for whom advice or treatment was sought by source of provider

Disparities in Care-seeking for Diarrhoea



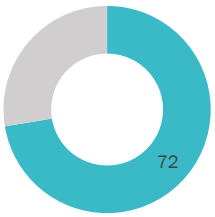
Percentage of children age 0-59 months with diarrhoea in the last two weeks for whom advice or treatment was sought at a health facility or provider

Feeding during Diarrhoea



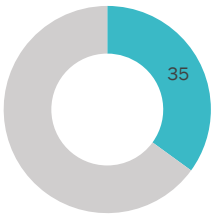
Percent distribution of children age 0-59 months with diarrhoea in the last two weeks by amount of liquids and food given during episode of diarrhoea

ORS Treatment for Diarrhoea



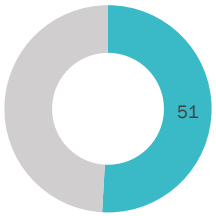
Percentage of children age 0-59 months with diarrhoea in the last two weeks treated with oral rehydration salt solution (ORS)

ORS + Zinc Treatment for Diarrhoea



Percentage of children age 0-59 months with diarrhoea in the last two weeks treated with oral rehydration salt solution (ORS) and zinc

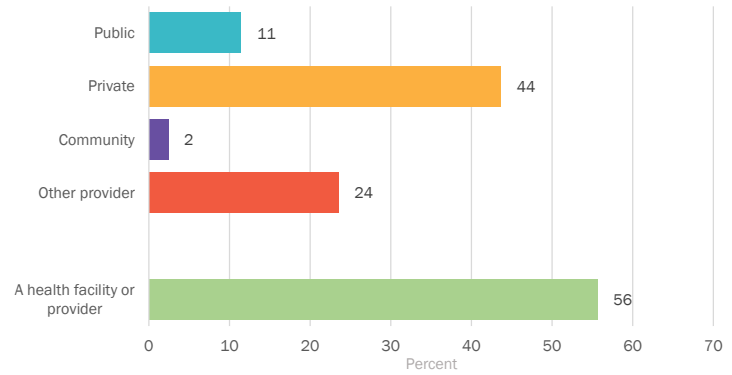
ORT + Continued Feeding for Diarrhoea



Percentage of children age 0-59 months with diarrhoea in the last two weeks who were given oral rehydration therapy (ORT) with continued feeding

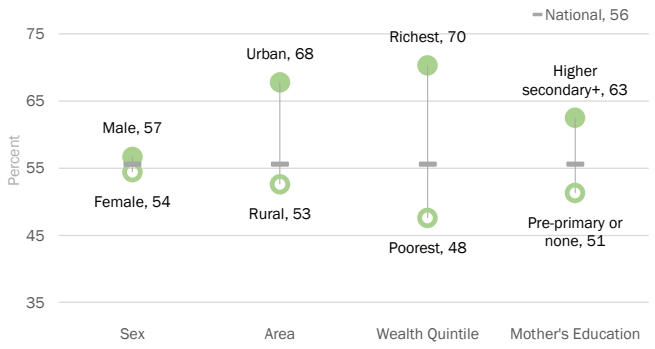
Care-seeking

Care-seeking during Fever



Percentage of children age 0-59 months with fever in the last two weeks for whom advice or treatment was sought, by source of advice or treatment

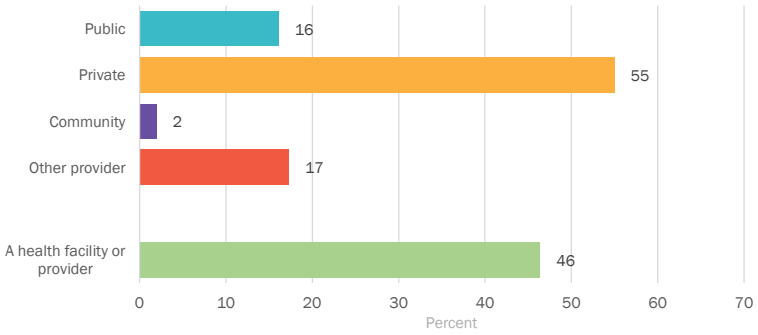
Disparities in Care-seeking during Fever



Percentage of children age 0-59 months with fever in the last two weeks for whom advice or treatment was sought at a health facility or provider

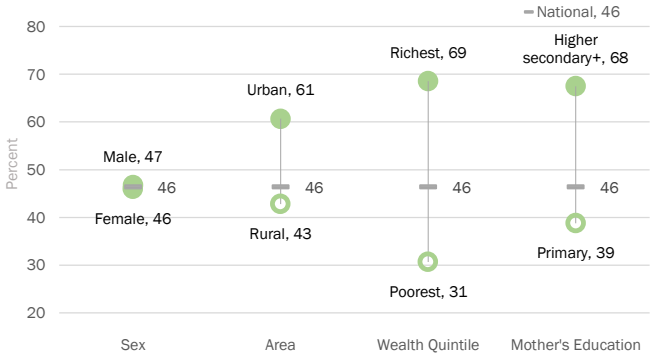
Symptoms of Acute Respiratory Infection (ARI)

Care-seeking for Symptoms of ARI



Percentage of children age 0-59 months with symptoms of ARI in the last two weeks for whom advice or treatment was sought, by source of advice or treatment

Disparities in Care-seeking for Symptoms of ARI



Percentage of children age 0-59 months with symptoms of ARI in the last two weeks for whom advice or treatment was sought at a health facility or provider

Divisional Data on Care-Seeking for Childhood Illness

Division	Care-Seeking at a health facility or provider for:		
	Diarrhoea	Fever	Symptoms of ARI
National	**30**	**56**	**46**
Barishal	24	51	54
Chattogram	26	59	59
Dhaka	32	67	56
Khulna	36	44	48
Mymensingh	26	51	18
Rajshahi	30	53	54
Rangpur	31	51	43
Sylhet	35	54	(*)

(*) Figures that are based on fewer than 25 unweighted case

The Bangladesh Multiple Indicator Cluster Survey (MICS) was carried out in 2019 by the Bangladesh Bureau of Statistics as part of the global MICS programme. Technical and financial support was also provided by the United Nations Children's Fund (UNICEF).

The objective of this snapshot is to disseminate selected findings from the Bangladesh MICS 2019 related to Child health & Care of Illness. Data from this snapshot can be found in table TC.2.1, TC2.2, TC.2.3, TC.2.4,TC.5.1 and TC.5.2

Further statistical snapshots and the Survey Findings Report for this and other surveys are available on mics.unicef.org/surveys.

Key Messages

- In recent years, significant progress has been made in Bangladesh to reduce child deaths from diarrhoea. Open defecation, in particular, is far less commonplace now than it was 20 years ago. But diarrhoea remains a leading killer of young children under five, especially in remote areas or humanitarian settings.
- Children aged 0-59 months who suffered from diarrhoea in the two weeks prior to the survey were more likely to be taken to a private care provider (44%) than any other type of provider. Only 30% of children who had suffered from diarrhoea taken to a health facility or provider. Similar results found for acute respiratory infection (ARI), where only 46% of children received treatment for ARI from a health facility, compared to 55% who take to a private provider. These results indicate that the use of private health providers, including community providers, is a common practice in Bangladesh.
- Of the children who had a fever in the two weeks prior to the survey, 56% were taken to a health facility or provider for care compared to 44% who received care from a private provider. Of these children, 26% received care from a community or 'other' provider. These data indicate that many children in Bangladesh continue to be taken to providers who are not regulated and from whom the quality of care is unknown.
- There are disparities in care seeking for the treatment of diarrhoea, fever, and ARI. Children from rural areas, poor households, or whose mothers/caretakers have no education are less likely to be taken to a provider for the care of diarrhoea, fever, and ARI. In some instances, the disparities are acute. For example, only 43% of children with symptoms of ARI in rural areas taken to a health facility or provider for care, compared to 61% of children in urban areas.
- Seventy-two percent of children with diarrhoea received ORS, but only 35% received ORS and zinc, which is the recommended treatment for diarrhoea for children aged 0-59 months. Just over half (51%) of the children who had suffered from diarrhoea received oral rehydration therapy and continued feeding.